Staff answers and cover notes

These are suggested responses to the invented case, not judgements about a learner’s health. Accept other safe, reasoned answers. All personal reflection is optional and private.

Wellbeing

How a person is doing in everyday life. It includes feeling supported, having things that matter to them, and being able to manage daily life or get help.

Mental health

A part of everyone's health, linked to thoughts, feelings and coping with daily life. Everyone has mental health. This does not mean everyone has a mental health condition.

Resilience

Adapting to a setback or change, recovering in your own time, and using support when needed. It can include rest, a different plan or asking for help. It never means having to put up with harm.

Invented case key

The learner uses people by planning a safe check-in, and downtime by choosing a quiet activity. An interest could also give purpose. Asking for help and adjusting the plan can show resilience. Sleep or safe accessible movement may be discussed as possible areas, but do not infer that the fictional learner has a sleep problem or require exercise. More than one supported link is valid.

Model and one realistic change

For the invented learner: during the next agreed break, try a short quiet making activity. Ask staff what materials are already available. If materials are not available, choose a quiet drawing or reading option. Review whether the step was useful at a later agreed check-in. The making activity is the one step. Staff contact is the support to arrange it; the alternative is a fallback. A change can make a helpful habit easier to keep by agreeing when or where it fits. It must not become five separate goals.

What the wheel means

This wheel is an authored reflection aid, not a test, validated scale, diagnosis, target score or measure of how healthy a person is. The five areas are broad reflection prompts. Uneven or blank areas do not establish risk, health or failure. Do not total, compare, display or interpret pupil wheels.

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Checks and tier guidance

Starter and model check

Everyone has mental health; this is not the same as everyone having a mental health condition. The invented learner shows resilience by using support and adjusting a plan. Ordinary upset can happen; do not diagnose from a feeling or classroom response.

Main hinge

A learner faces a setback. Which response can show resilience? Correct B: They can pause, adapt the plan or ask for safe support. Check privately by words, pointing or usual aid. If B or a safe equivalent with a reason, proceed. If A, unclear/no response, re-model supported adaptation and use the parallel check. Pass is not a wrong answer.

Parallel hinge

What can a wellbeing wheel tell us? Correct A: It can prompt reflection and one possible next step. The shape does not diagnose a condition.

Supported

Use the same age-respectful five-domain wheel. Offer the word bank, two example choices, a sentence frame and a completed fictional example. Pointing, drawing, usual aid or exact scribe/readback valid. Choose one realistic step with support and record content prompts.

Independent

Use the glossary, wheel prompts, visible model and usual aids. Explain the fictional example, privately choose one realistic change or fictional equivalent, and identify needed support or a barrier. Record actual independent choices and any prompts honestly.

Aspire

Keep all normal aids. Evaluate two plausible steps for the invented learner, explain the more realistic choice, and adapt it for a barrier without blaming the learner. Still choose only one step. Extra personal disclosure, more changes or longer writing are not required.

Realistic step check

The step is small enough, fits access and available resources or names needed support, and has a time/cue and fallback. No spending, sleep target, exercise or family contact is required. A barrier is context to adapt around, not personal failure. Each learner ends with one choice, even when Aspire compares two possibilities.

Self assessment

Ready to explain / With a prompt / Need another example describe learning only. All boxes start blank. Do not convert these to mood, symptom or health scores. Pass or fictional explanation can be used. Private personal goals need not be read to assess understanding.

Misconceptions

Resilience means never feeling upset or putting up with harm. Feelings can vary. Resilience can include rest, support, safe boundaries and changing a plan or situation.

A wheel score or one feeling proves a mental health condition, and a small habit guarantees a fix. Everyone has mental health. This wheel has no diagnostic score. A small step may help, while support can still be needed.

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Safe delivery and evidence

Discreet supervised step out

Before the lesson, quietly agree the familiar pause signal and supervised place/route with the learner. A pupil may show a pause card or use the usual aid without a public explanation. A staff member acknowledges and supports the agreed route; never send a learner off alone or remove supervision. Offer a neutral task and a supported return, or arrange a later private catch-up. Pass/step-out is not failure and does not require personal disclosure.

Privacy and safeguarding

Any disclosure goes to the safeguarding lead through school procedure. Listen calmly, reassure, do not probe, investigate or promise secrecy. Follow urgent school procedures where needed. Keep actual concerns and identifying details out of the lesson pack, book tasks and shared learning evidence; use the school's separate safeguarding record. If a learner cannot identify a trusted adult, support them privately and do not press. Private wheels/plans are not collected, photographed, displayed or compared for assessment. Scribing a personal card is only by agreement; the learner keeps it.

Help route

Support may come from a trusted adult, school staff, a family doctor or a suitable helpline. Ask trusted school staff for help to find an age-appropriate route. Seek help if a concern persists, worsens, affects everyday life or feels unsafe; you do not need a diagnosis first. If the first adult is unavailable or unhelpful, try another trusted adult through the agreed route. For immediate concern, get school staff straight away and use the school's urgent procedure.

Evidence and marking

Staff Evidence Sheet records explanation of the three ideas, an appropriate fictional wheel-area link, a realistic fictional step with support/barrier, learning self-check if offered, usual mode/prompts, one NS and own response. No personal wheel ratings, plan details, health/family history, names or disclosures. Private personal change remains the learner's choice; fictional evidence is equally valid. Do not infer health status, improvement or diagnosis from learning performance. VF verbal feedback; WS worked with support; I independent; NS next step in green; // self-edit; ? read this back to me; E actual evidence banked; R responded. R only when Voice and Audience are present, one NS is set and the pupil responds in the same colour, on the same page or usual mode. Offer to the Communication portfolio; staff confirm any mapping.

Source basis

Current statutory secondary health-education guidance, revised in 2025 and in force from September 2026, and public-health information on universal mental health, practical wellbeing and support routes were checked for this narrow lesson. These support the generic teaching statements. The lesson does not cover the whole statutory curriculum. The wheel is newly authored, not a validated instrument. Detailed source descriptors are in the staff facts record.

Scope

Thursday 7 January 2027 P4, forty minutes, Year A, LAUNCH pair aged 14–16, formative Week 1. Owner brief supplies row 148 and exact outcome; workbook not reopened. Week 1 only. No later lesson content.

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